

1. Administrative & Study Identification

Full Official Title: A Phase 1/2 Study of the Safety, Tolerability, Pharmacokinetics and Preliminary Efficacy of Relatlimab Plus Nivolumab in Pediatric and Young Adult Participants with Recurrent or Refractory Classical Hodgkin Lymphoma and Non-Hodgkin Lymphoma **Study Title:** A Study to Evaluate the Safety, Tolerability, Drug Levels, and Preliminary Efficacy of Relatlimab Plus Nivolumab in Pediatric and Young Adults With Hodgkin and Non-Hodgkin Lymphoma **Short Title/Acronym:** RELATIVITY-069 **Protocol Number:** CA224-069 **NCT Number:** NCT05255601 **Posting ID:** 342032146445767089 **Sponsor/Company Name:** Bristol Myers Squibb **Investigational Product:** Relatlimab plus Nivolumab (Fixed-dose combination; Trade Names: Opdualag, Opdivo) **Phase of Development:** PHASE1, PHASE2 **Trial Status:** RECRUITING **Medical Monitor:** Head of the Global Submission Management - Clinical Trials (Bristol Myers Squibb)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and define the Maximum Tolerated Dose (MTD) or Recommended Phase 2 Dose (RP2D) for the combination of relatlimab plus nivolumab in pediatric participants with R/R cHL and NHL, and to assess the pharmacokinetics (drug levels) of the combination. **Secondary Objectives:** To evaluate preliminary efficacy in the expansion cohorts, and to monitor the incidence of adverse events (AEs), serious adverse events (SAEs), and AEs leading to discontinuation.

2.2 Background & Rationale While outcomes for Non-Hodgkin Lymphoma (NHL) and classical Hodgkin Lymphoma (cHL) at diagnosis are favorable with tailored standard treatments, pediatric and young adult patients with recurrent or refractory (R/R) disease face poor prognoses and lack standardized treatment recommendations. Nivolumab targets the PD-1 pathway, and relatlimab is a novel immune checkpoint inhibitor targeting the LAG-3 protein. By combining these therapies, the treatment aims to target distinct pathways to prevent cancer cells from evading the immune system, potentially yielding a more effective and durable clinical response.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Part A: Dose-escalation/safety; Part B: Expansion cohorts) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Targeted Enrollment: 68 participants **Number of Sites:** Approximately 50 centers across 7 countries **Estimated Study Start:** 2022 **Trial Status:** RECRUITING

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participants with pathologically confirmed high-risk recurrent or refractory classical Hodgkin lymphoma (R/R cHL), after non-response to or failure of 1 or more lines of standard therapy. 2. Participants with pathologically confirmed R/R NHL after non-response to or failure of 1 or more lines of standard therapy, including, but not limited to, R/R primary mediastinal B-cell lymphoma, diffuse large B-cell lymphoma (DLBCL), mediastinal gray zone lymphoma (MGZL), anaplastic large cell lymphoma (ALCL), or peripheral T-cell lymphoma (PTCL). 3. Participants with pathologically confirmed R/R NHL after non-response to or failure of 2 or more lines of standard therapy, including Burkitt lymphoma (blast count < 25% malignant Burkitt cells and/or per the investigator's clinical assessment of risk status), lymphoblastic lymphoma (blast count < 25% of marrow nucleated cells and/or per the investigator's clinical assessment of risk status), NK/T-cell lymphoma (nasal and non-nasal NK/T-cell lymphoma subtypes, but not aggressive NK/T-cell leukemia/lymphoma subtype). 4. The participant's current disease state must be R/R to standard therapy. 5. Participants must have measurable PET positive disease in both cHL and NHL cohorts.

4.2 Exclusion Criteria 1. Primary CNS lymphoma of the brain or spinal cord, and secondary CNS lymphoma (ie, from systemic non-Hodgkin lymphoma) involving the brain, spinal cord, or with leptomeningeal seeding. 2. Prior treatment with an anti-cytotoxic T-lymphocyte-associated protein 4 (anti-CTLA-4) antibody, or any other antibody or drug specifically targeting T-cell co-stimulation or checkpoint pathways, with the exception of anti-PD(L)-1 targeted therapies. 3. Prior treatment with lymphocyte activation gene-3 (LAG-3)-targeted agents. 4. Participants with clinically significant systemic illnesses unrelated to the cancer as judged by the investigators, which would compromise the participant's ability to tolerate the study treatment. 5. Participants with autoimmune disease. 6. Prior allogeneic bone marrow transplantation.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Part A utilizes escalating doses of relatlimab and nivolumab to identify the optimal dose. Part B utilizes the selected dose of the nivolumab and relatlimab combination. **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Up to a maximum of 2 years of study treatment (unless discontinued due to progressive metabolic disease, unacceptable toxicity, or patient withdrawal).

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care per institutional standards. **Prohibited:** Use of other experimental therapies, LAG-3 targeted agents, or anti-CTLA-4 pathway inhibitors outside of the protocol specifications (except anti-PD(L)-1 therapies).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	2 to 4 weeks prior	Informed Consent, Medical History, Baseline Scans (PET), Eligibility check
2	Day 0	First Dose (Part A or Part B), Safety and Pharmacokinetic Labs
3	Treatment Phase	Ongoing administration of Relatlimab + Nivolumab; AE/SAE monitoring, Efficacy Scans (approx. up to 2 years)
4	Follow-up	Safety monitoring and survival follow-up up to 135 days following last dose and further survival tracking post-treatment

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes

- Incidence of dose-limiting toxicities (DLTs):** DLT evaluation window is 4 weeks from start of treatment. Safety evaluation will continue up to 135 days following last dose.
- Maximum tolerated dose or Recommended phase 2 dose (MTD/ RP2D).**
- Number of participants with Adverse Events (AEs).**

7.2 Secondary Outcomes

- Number of participants with AEs.**
- Number of participants with Serious Adverse Events (SAEs).**
- Number of participants with AEs leading to discontinuation.**

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection evaluating overall safety, tolerability, and clinical laboratory abnormalities related to the checkpoint inhibitor combination. Safety evaluation continues up to 135 days following last dose. **Serious Adverse Events (SAEs):** Documented continuously to monitor for immune-mediated toxicities or severe reactions. **Data Monitoring Committee (DMC):** Internal/External safety monitoring

boards managing the dose-escalation phase (Part A) transitions to the expansion phase (Part B).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all participants receiving ≥ 1 dose of the combination) and Efficacy Set. **Sample Size**

Justification: The targeted total of 68 participants is powered to establish safety, tolerability, and drug levels (MTD/RP2D) and to detect preliminary efficacy signals. **Handling of Missing Data:** Standard survival and time-to-event methodology (e.g., Kaplan-Meier estimates); patients lost to follow-up censored at the date of last contact.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine clinical, safety, and radiologic monitoring throughout the treatment phase and specifically up to 135 days post-treatment for safety evaluations. **Audit Trail:** eCRF locking and standard Sponsor quality assurance compliance protocols for investigational new drugs.

11. Study Identifiers & Governance

Primary ID: CA224-069 **Registry Number:** NCT05255601 **Posting ID:** 342032146445767089 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** A Study to Evaluate the Safety, Tolerability, Drug Levels, and Preliminary Efficacy of Relatlimab Plus Nivolumab in Pediatric and Young Adults With Hodgkin and Non-Hodgkin Lymphoma **Official Regulatory Title:** A Phase 1/2 Study of the Safety, Tolerability, Pharmacokinetics and Preliminary Efficacy of Relatlimab Plus Nivolumab in Pediatric and Young Adult Participants With Recurrent or Refractory Classical Hodgkin Lymphoma and Non-Hodgkin Lymphoma

12. Clinical Framework (Oncology Specific)

Disease Areas: Lymphoma, Non-Hodgkin, Hodgkin Disease (Preferred UMLS Name: Lymphoma) **Primary Condition:** Recurrent or Refractory Classical Hodgkin Lymphoma (cHL) and Non-Hodgkin Lymphoma (NHL)

Diagnosis Threshold: Measurable PET-positive disease; R/R status post 1 or 2 lines of standard therapy depending on subtype

Medical Methodology: Dual Immune Checkpoint Inhibition (PD-1 + LAG-3) **Optimization Target:** Maximum Tolerated Dose (MTD) / Recommended Phase 2 Dose (RP2D)

13. Study Procedures & Methodology

Management Pathway: Two-part sequential pathway (Part A: Dose Escalation & PK validation → Part B: Dose Expansion)
Education Protocol (HCP): Site initiation and training on immune-related adverse events (irAEs) management protocols. **Education Protocol (Patient):** Informed consent discussions encompassing the investigational nature of dual-checkpoint inhibitors. **Quality Audit Mechanism:** Independent central review of PET scans for objective response criteria; continuous Sponsor medical monitoring.

14. Observation & Follow-Up Schedule

Total Duration: Up to 5 years (2 years of active treatment phase + robust safety tracking up to 135 days post-dose + further survival follow-up). **Onsite Visit Cadence:** Routine administration cycles (length dependent on chosen RP2D schedule) with scheduled re-staging scans. **Remote Monitoring Frequency:** During the follow-up period per protocol. **Checklist Review Interval:** Regular cycle-by-cycle safety assessments prior to each subsequent infusion.

15. Sign-off & Approval

Role	Name	Signature	Date						
Principal Investigator	Nobuko Hijiya, MD (Columbia Site)	/							
	[Name]		03-Apr-2026		Clinical Operations Lead		[Name]		
			03-Apr-2026		Lead Statistician		[Name]		03-Apr-2026